AC-14 LVAD Emergency

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for a patient with a durable left-ventricular assist device (LVAD). A continuous-flow LVAD may not produce a palpable pulse; assess perfusion and device function together.

Perfusion-first LVAD emergency pathway

1. Assess perfusion, not pulse alone	Mental status, skin temperature/color, capillary refill, BP/MAP when obtainable, ETCO2 if ventilated, breathing, and signs of shock.
2. Device check in parallel	Listen for hum, inspect controller alarms/connection, power source, driveline, and spare equipment. Ask patient/caregiver to assist when possible.
3. Adequate perfusion	Treat the presenting complaint, continuous monitoring, 12-lead when indicated, and contact the implanting center/LVAD coordinator early.
4. Poor perfusion	Correct simple power/connection problems and seek device guidance while treating reversible causes. For hypotension, use cautious fluid if not overloaded and Medical Control as needed.
5. No effective perfusion	Begin standard CPR without delaying for prolonged device troubleshooting when the patient is unresponsive with absent/ineffective circulation. Follow AC-03/AC-09.
6. Transport	Bring controller, power modules/batteries, spare equipment, and caregiver when feasible. Route to a device-capable center under local plan.

Key treatment and handoff

Item	Approved action
Electrical therapy	Use standard EMS defibrillation/cardioversion/pacing when clinically indicated; keep pads clear of device hardware and follow device-center guidance.
Bleeding	Control external hemorrhage and anticipate anticoagulant/antiplatelet therapy until verified otherwise.

A nonpalpable pulse alone is not evidence of LVAD arrest. Conversely, do not withhold CPR from a truly unresponsive patient with absent/effectively absent perfusion while waiting for device support.

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SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Do not disconnect a driveline, turn off a functioning controller, or disconnect two power sources simultaneously.

- Avoid compression/traction on the driveline and protect equipment during movement.

- Contact the implanting center/LVAD coordinator early; patient-specific device instructions take priority for device operation.

Linked pathways

Pathway	Use when
AC-03	Adult cardiac arrest
AC-09	VF/pulseless VT
AC-04	ACS/STEMI
AM-05	Hypotension/shock

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.